

Eliminating the Potential Roots of Fear: Addressing Thanatophobia in Childhood

Travis Gray & Jaclyn Maass

University of Central Oklahoma, Edmond, OK

Abstract

The fear of death, also referred to as thanatophobia or death anxiety, is one of the most common fears among people. Terror Management Theory suggests that a psychological conflict creates a debilitating anxiety in people, which can lead to seeking forms of immortality to deny death. After reviewing the literature on the topic, we have identified and will focus on three key factors related to thanatophobia: culture, gender, and age. Based on this research, we propose that thanatophobia may evolve based on how death was handled during childhood.

Keywords: Thanatophobia; Gender; Culture; Age; Childhood

Eliminating the Potential Roots of Fear: Addressing Thanatophobia in Childhood

Thanatophobia: The Fear of Death

While death is an inevitable outcome of life and people typically have repeated exposures or experiences with it, there is still much fear and anxiety surrounding the topic. This fear is based largely on the unpredictability of death and a lack of understanding of what follows death. These are only two of numerous aspects shrouding death that may eventually transform into a fear of death, otherwise known as thanatophobia or death anxiety. This is a complex issue that can be a source of psychological and existential distress (Blomstrom et al., 2022; Lehto & Stein, 2009; Kesebir, 2014). On a strict definitional basis, thanatophobia, also commonly referred to as death anxiety, has often been conceptualized as a feeling of dread, apprehension, or fear in response to death or dying (Alagh & Chaturvedi, 2022; Blomstrom et al., 2022). Fear of death itself is likely not only restricted to one's own demise. It may extend to the loss of others, such as family, friends, and pets. Thus, thanatophobia may be internalized (e.g., the fear of one's own death) or externalized (e.g., the fear of losing loved ones).

This article seeks to review the theory behind thanatophobia and ultimately propose methods to combat its formation in childhood. We will also review individual differences that can shape the formation and manifestation of the phobia, including a person's culture, gender, and age. These are important factors to take into account when treating thanatophobia. We will also review therapies for thanatophobia that have worked for adults and offer some therapeutic suggestions for dealing with thanatophobia in children. Before examining these potential factors related to the origin and treatment of thanatophobia, an assessment of terror management theory (TMT) provides a theoretical foundation as to why death anxiety exists.

Organized Fear: Terror Management Theory

Thanatophobia is recognized as the anxiety and trepidation that spawns from the uncomfortable recognition of death (Blomstrom et al, 2022; Lehto & Stein, 2009; Kesebir, 2014), which can be better understood through

We have no known conflict of interest to disclose.

Correspondence concerning this article should be addressed to Travis Gray, M.S., University of Central Oklahoma, Edmond, United States. tgray13@uco.edu

Terror Management Theory (TMT; Greenberg et al., 1986). According to TMT, a psychological conflict is created through a human's instinct of self-preservation combined with the understanding that one will perish, and that is this psychological conflict that produces the terror of death anxiety. The theory suggests that the finality of death itself is a large factor in what creates the motivation for humans to live. Moreover, as humans develop and possess the ability of self-awareness, the recognition that death may occur anytime and anywhere may generate debilitating apprehensions about death (otherwise referred to as thanatophobia or death anxiety). In response to these apprehensions, humans cope by seeking immortality through symbolic (e.g., achievements) or literal (e.g., afterlife, reincarnation) means. TMT spawned primarily from the work of Ernest Becker, whose *The Denial of Death* has been an extensively studied, supported, and predominantly accepted as a theory that explains why individuals suffer from death anxiety (Greenberg et al., 1986; Iverach et al., 2014; Zuccala & Abbott, 2021). Therefore, it is evident that humans have potential options to defend against thanatophobia. However, the examination of culture, gender, and age, which all appear to be related to thanatophobia, is still required. While the experience of death is universal, there are several individual differences in how death is viewed, experienced, and managed.

Thanatophobia and Culture

Numerous articles that have examined thanatophobia include a variety of diverse samples. Thus, similar to how age and gender may each play a separate or a combined role in relationship with thanatophobia, a factor that also resides someplace within all examined studies is culture itself. And a principal justification to examine culture is that this factor will be present in any study, regardless of context.

A study examining religion, contact with the dead, and death anxiety in Mexican American populations reported that participants who felt more grateful to God experienced lower levels of death anxiety (Krause & Bastida, 2012). A common Mexican tradition involves celebrating the dead, usually referred to as *Día de los Muertos* (The Day of the Dead). It is a time to remember, reflect, and celebrate the life of passed loved ones through flowers, candles, food, and cemetery visitations. Moreover, foods often are themed around aspects of death, such as skulls and bones. Nonetheless, at the core of this celebration, the living and dead are brought together once more. Krause and Bastida (2012) mention that feelings of connectedness, beyond just familial ties, increase as perceived contact with the dead increases. This suggests that the more contact one has with the dead, the more one is able to recognize that everyone shares a connectedness. Moreover, those who held higher feelings of connectedness felt a greater gratefulness to God. This leads to the finding that those who felt more gratefulness to God experienced lesser death anxiety. However, one may question if it is the culture or the religion that is responsible for the reduction of death anxiety.

To further examine this relationship between culture, religion, and thanatophobia, a study by Schumaker and Warren (1991) examined Japanese and Australian cultures and found that Japanese participants reported higher thanatophobia scores overall in comparison to Australian participants. They noted that Eastern cultures may elect to view death as a transition into a new existence, whereas Western cultures tend to view death of the body is equal to a death of the self. This is likely in part due to the difference in the views of religion across cultures. One hypothesis for Japanese participants having higher death anxieties than Australian participants was the relationship between religion and death anxiety. As Schumaker and Warren (1991) note from an international Gallup survey, Japan was found to have the lowest rate of religious belief and religious practice of the surveyed countries (for list, see Naroll, 1983 as cited in Schumaker & Warren, 1991). The survey showed that only 14% of Japanese respondents considered religion as very important and only 19% believed in religion in general. And Australians reported 25% and 38.3% respectively. Other comparisons of Western and Eastern cultures showed even wider gaps, with respondents in America reporting 30% less emphasis on religion and 21% less belief in religion compared to respondents in India. It should be noted that the difference in thanatophobia rates between cultures in Schumaker and Warren's (1991) study was driven by the male participants. Women participants did not differ in thanatophobia between cultures. This may suggest that gender supersedes culture as a predictive factor of thanatophobia.

Although they are intricately connected, religion and culture are not synonymous. So, while a lack of religious beliefs may be a factor in the higher levels of thanatophobia when examining culture, other factors remain which are likely shaping a culture's view of death. For example, historical context between cultures is a critical factor. One such example is the potential ramifications from the atomic bomb droppings that Japan suffered in 1945. Prior to the atom bombs, institutionalized suicide was seemingly seen as honorable, evidenced by kamikaze pilots or committing seppuku. While Schumaker and Warren conducted this study in 1991, the repercussions from the annihilation of two Japanese cities may still be a partial aspect to the higher reported levels of thanatophobia. Still, when examining the overall relationship between culture and death anxiety likely requires further analysis. As

aforementioned, culture may simply be too broad to observe as a whole. Thus, when examining thanatophobia, culture may require division into more specific aspects, such as historical context, religion, social structure, language, etc. However, the relationship between thanatophobia and gender still requires an investigation.

Thanatophobia and Gender

While both men and women suffer from various levels of thanatophobia, women typically report higher levels of thanatophobia than men (Furer & Walker, 2008; Sinoff, 2017). Seminal research by Russac et al. (2007) found that men and women have peak death anxiety in their early twenties, with a steady decrease over time. However, women may have an additional spike in death anxiety around age 50. This research utilized three measures of death anxiety across two studies, measuring four aspects of death anxiety: death of self, dying of self, death of others, and dying of others.

However, the research had issues with adequately representing men, individuals under 20, and individuals over 60 in their samples. The samples of both studies consisted primarily of university students and local community church volunteers, which are vastly different populations. Further, this study exclusively sampled from white, educated, industrialized, rich, and democratic (“WEIRD”) samples. While such a sample can limit the generalizability of research, there is evidence from other research to suggest that these results do extend to non-WEIRD populations. For example, research by Suhail and Akram (2002) examined death anxiety, gender, age, and religiosity in a Pakistani population. Here, younger women (aged 16-30) reported higher death concerns than younger men across multiple measures. Women reported greater apprehension about various dimensions of death, such as shortness of life, fear of not being, and disintegration of one’s body after death. Although they used a non-probability convenience sample, Suhail and Akram (2002) recruited participants from various backgrounds (universities, varying workplaces, and homes) to increase external validity.

Further support for the relationship between thanatophobia and gender was found in a recent study utilizing a young Turkish sample (Çakar, 2020). This research found that in a sample of Turkish university students (current and graduated ranging in age from 20-36, $M = 23.66$), women had significantly greater death anxiety than Turkish men. This study extended the focus on death anxiety to include measures of meaning of life (including presence of meaning of life and search for meaning in life), and loneliness. While death anxiety was positively related to loneliness and negatively related to meaning in life, it is noteworthy that among these measures, the only factor that varied significantly by gender was death anxiety. This further suggests that thanatophobia is related to gender. However, the exact specifics of that relationship may yet to be discovered. Like the previously examined studies, Çakar (2020) utilized a more highly educated sample. Most of the studies discussed thus far have focused on more highly educated samples.

However, Schumaker and Warren (1991) examined death anxiety in Japanese and Australian populations, with each sample including individuals from a broad range of backgrounds, education, and socioeconomic status levels. The results continued the trend and found that Australian women scored significantly higher in death anxiety than men. While the research traditionally shows that women tend to report higher levels of thanatophobia than men, some studies have found men to report higher levels of thanatophobia than women. One example is Chopik (2017), who examined death related thoughts, death anxiety, social support, and health. In a longitudinal study of 9,815 adults over a four-year duration, results showed that men reported higher levels of death anxiety than women. However, upon closer analysis, one may question this finding. Death anxiety was assessed by a single question as part of a larger questionnaire which assessed anxiety more broadly. The question asked how often over the previous week one experienced a fear of death, which was answered on a 4-point scale from 1 (never) to 4 (most of the time). With such narrow stimulus sampling, it is unclear if this result would be replicated using the validated measures used in the previously discussed studies.

While many of these studies found a relationship between death anxiety and gender across multiple cultures, this was not ubiquitous. Schumaker and Warren (1991), who sampled from Japanese and Australian populations, did not find a relationship between gender and death anxiety in their Japanese sample. Moreover, Chopik’s (2017) study may lack in its measure of thanatophobia to conclude that men suffer from greater levels of thanatophobia. Nonetheless, gender may have adequate potential to serve as a gauge to measure thanatophobia, but age may serve as an even better one.

Thanatophobia and Age

Age is likely one of the strongest factors that has been consistently observed to relate with thanatophobia. Two critical aspects that age brings are life experience and exposure to death. There are relatively consistent developmental changes in how death can be comprehended based on age, allowing family, practitioners, and researchers to be more consistent in pinpointing where someone stands in their comprehension of death and potential development of thanatophobia. Age is also ubiquitous across cultures and genders. Many of the studies previously discussed have investigated differences in thanatophobia across different age ranges, as well as between genders or cultures.

Research is fairly consistent in showing greater levels of thanatophobia in younger populations compared to older populations. Of the previously discussed studies, Russac et al. (2007), Sinoff (2017), and Chopik (2017), all found a negative relationship between age and thanatophobia. For example, Russac et al (2007) found that men and women have peak death anxiety in the early 20s, and this fear steadily declines over time. In a three-tiered study examining children's inability to understand parental death anxiety, Sinoff (2017) noted that children reported higher death anxiety scores than their parents. Moreover, the child's proxy assessment of a parent's death anxiety was even greater (twice as high) compared to actual reported scores of adults. It was noted that children would project their own fears of death upon a parent. Observing the responses, children reported fears regarding death itself, whereas parents reported fears about the process of dying. For example, the children would report beliefs that a parent feared death, feared meeting death before a child, or felt that the future was bleak. But, in reality, these beliefs correlated with the children's own fear of death rather than the parents' (Sinoff, 2017). These findings suggest that thanatophobia does change as one ages. Particularly, death anxiety appears to adjust over time, with age serving as a measurement of transitioning from fearing death itself to fearing the process of dying.

In further potential support of age leading to transitioning thoughts about death anxiety, Mohammadpour et al. (2018) examined the relationships between perception of aging, demographic factors, and death anxiety in Iranian populations (age $M = 73.97 \pm 7.68$). Perception of aging included five dimensions: overall positive consequences of aging (e.g., wiser, more appreciative), positive changes to control of one's life, overall negative consequences (including loss of control), having a constant or more cyclical awareness of one's age, and negative emotional responses to aging. Based on their data, death anxiety was significantly predicted by age and all dimensions of the perception of aging scale, except for views of negative consequences. Simply put, a realistic perception and acceptance of aging was found to correlate with having lower levels of death anxiety. The older adults reported less anxiety when accepting the terms of aging, despite how close they may be to death from age, physical ailments, or mental ailments (Mohammadpour et al., 2018).

Two potential issues of this study should be noted. The first issue is that this study may not be fully supportive of age serving as a catalyst to changes in thanatophobia over time. The sample only included people over the age of 60. However, other research has included a wider range of ages to further establish this relationship. For example, one study examined levels of anxiety, depression, and death anxiety in junior high students (age $M = 13.08$), high school students (age $M = 16.29$), and adults (age $M = 30.47$; Koocher et al., 1976). Across the three measures, there was an overall trend of increases (in depression, anxiety, and death anxiety) from junior high school students to high school students, but then a decrease from high school to adulthood (although not all differences were significant). The decrease from high school students to adults was only statistically significant for anxiety. However, we believe the lack of statistical significance in the quantitative results should be interpreted with caution as there was a large difference in group sizes across the three age ranges (middle school, $n = 75$, high school, $n = 111$, adults, $n = 38$). The variation of the groups may have created other issues, such as difference in variability between groups. The researchers do not explicitly report a violation of homogeneity of variances (or lack thereof), so it is unclear whether the sample size differences would have impacted the statistical significance.

The researchers suggest that the overall trend of changes in anxieties and depression may involve the emotional, mental, and physical alterations that occur around this age. Specifically, the increase in anxieties and depression in high school aged children may be due to leaving child-like understandings of death behind and adopting a more realistic, but also exaggerated, understanding of death. However, as we continue to age, people begin to accept aging and death, seeing it as the more multifaceted reality that it is.

Further, investigating decreases in thanatophobia into adulthood, Chopik (2017) examined younger, middle, and older-aged adults' death anxiety and found that older adults reported less death anxiety than both younger and middle-aged adults. More importantly, this decline of death anxiety over time was found after controlling for self-rated health, chronic illnesses, and gender (Chopik, 2017). While Chopik's (2017) study suffers from a relatively weak measure of death anxiety, the finding of older adults reporting lower levels of death anxiety

appears to be predominant and recurring (e.g., Alagh & Chaturvedil, 2022; Furer & Walker, 2008; Iverach et al., 2014; Mohammadpour et al., 2018; Sinoff, 2017). Taken together, there appears to be a relatively consistent finding that thanatophobia decreases with age, regardless of health and gender.

We must note that there are studies which reported an increase of thanatophobia with age within Pakistani populations (Suhail & Akram, 2002) as well as Japanese and Australian populations (Schumaker & Warren, 1991). Thanatophobia is likely undergoing continual change at an individual scale, as concerns about death differ at different ages or in specific life contexts (Furer & Walker, 2008). Despite this, these changes in one's thanatophobia are still likely to be more predictable and more consistent based on a measurement of time (e.g., Mohammadpour et al., 2018; Russac et al., 2007) rather than on differences in gender (e.g., Chopik, 2017) or culture. Therefore, age should remain an important factor when examining thanatophobia. Age provides a more objective criterion to observe potential levels of death anxiety than gender and culture, as a person who is 17 years old will be the same age across any population.

Thanatophobia in Children

One's comprehension of and reaction to death begins early in life and may remain with a person for the rest of their life (Dickinson, 1992; Knight et al., 2000). Children often view the idea of death differently than adults (Mitchell & Schulman, 1981). A child who does not understand death will often create a reality which best alleviates or allows some control over the horrors of death. For some children, this may include sayings, such as "you step on a crack, you break your mother's back." It may also include other mechanisms, such as prayers or animism (Mitchell & Schulman, 1981). Still, these are often unrealistic defensive measures which protect children until maturation allows them to understand the complexities of death and repress the unfavorable feelings that can accompany such understanding (Mitchell & Schulman, 1981).

A potential reason for these unrealistic defense mechanisms is that children do not fully understand the dimensions of death (irreversibility, finality, causality, and inevitability), which hinders the ability to properly accept and cope with death (Schonfeld, 1993). Irreversibility is the recognition that death is permanent and cannot be undone. Finality is the recognition that death brings a ceasing of all life functions. For example, one may bury food with a deceased pet, but it should be evident that the deceased pet will not consume the food. Causality is the recognition of the actual cause of death. Death does not occur without some physical reason or cause, but children often apply a form of "magical thinking" when regarding death. For example, many children believe in immanent justice, which dictates that the good will be rewarded and the bad will be punished (Schonfeld, 1993). Thus, when applied to a scenario of death, it is possible that a child may believe someone died as a form of punishment for evil or wrongdoing. Inevitability is the recognition that death happens to everything that is alive. Researchers differ in their estimations of when children are most likely to develop understanding of these dimensions of death. The estimate has increased over the years, with Schonfeld (1993) initially suggesting that children typically recognized these dimensions between ages 5 to 7, Knight (2000) reporting that the average first encounter with death occurred between ages 7 to 9, and more recently, Sinoff (2017) suggesting that children do not appear to recognize the finality of death until ages 9 to 10.

In an effort to understand people's initial experiences with death, Dickinson (1992) conducted research with college students ($n = 440$; age $M = 23.79$ years), who wrote essays detailing their first experience with death. The average age of a first death experience was 7.95 years old. Fifty-seven percent of all first death experiences involved a relative, while 28% included a pet. Participants reported that at the time of death, their emotional reactions were similar to how adults would act: relief, confusion, disbelief, fear, happiness, anger, sadness, emptiness, shock, and guilt (Dickinson, 1992). However, other researchers posit that a true understanding of death is not established until age 10 (Sinoff, 2017). This may suggest that reactions to death are seemingly innate, as if children recognize the burden of death, despite not fully understanding it (as mentioned by Dickinson, 1992; Schonfeld, 1993). It could also be indicative that the college participants were retroactively applying their current understanding of death rather than reporting on a true assessment of how they would have labeled their feelings at that earlier age.

Knight et al. (2000) replicated Dickinson's (1992) work with a similar sample and found a positive relationship between a person's reaction to their first encounter with death and their current view of death. Both studies found that the majority of first death experiences involved a human (primarily grandparents, but sometimes a sibling or parent) and a smaller proportion involved a pet. The type of loss is important to reference when examining thanatophobia, for how an adult responds to a loss may dictate how a child responds to death later in life. Knight et al.'s (2000) study found that parents' discussion with children about death was related to different current feelings

toward death, depending on whether the discussion concerned the death of a pet or a person. Those whose first death encounter (and therefore first parental discussion concerning death) was for a pet had a lower current fear of death than the respondents whose first encounter with death was after the loss of a person. Knight et al. (2000) detail two potential reasons for this. First, the loss of a pet is likely less traumatic than the loss of a person for the parents. In having less grief, the parents and children were able to have a discussion regarding death without the potential for grief denying children access to knowledge about death, particularly any questions that a child may have regarding death. The loss of a person may have caused anguish too great for the adults, which could result in denying knowledge or conversation about death to the child. This highlights the importance of being able to discuss death with children in a healthy manner, as these feelings and interactions with death can have lasting consequences. More broadly, parents who experience extreme and reclusive responses to death and dying may lead to children's needs being unmet. This could take the form of failure to answer questions about death, denial of hospital visitation of a dying person, or potential change in routine, such as reduced maternal feeding or reduced attention to a child (Dickinson, 1992; Schonfeld, 1993).

Thanatophobia Therapy for Youth

There appears to be a limited number of empirical studies that specifically examine thanatophobia in children (e.g., Koocher et al., 1976; Sinoff, 2017), and fewer studies that examine therapy styles for reducing thanatophobia in children (e.g., Seidi & Jaff, 2023). For bereaved children, the lack of available empirical studies on effective, positive-outcome interventions is an evident hurdle (Chen & Panebianco, 2018).

In adult populations, cognitive behavioral therapy (CBT) appears the predominant choice to combat thanatophobia in reviews (e.g., Furer & Walker, 2008; Iverach et al., 2014) and empirical studies (Moradi et al., 2022; Roza et al., 2022). CBT is also being utilized to provide a form of online self-therapy to combat thanatophobia as a more affordable alternative to in-person therapy (Menzies et al., 2021). Aside from CBT, other therapy styles used to ease thanatophobia include exposure (Furer & Walker, 2008; Iverach et al., 2014) and narrative therapies (Nozari et al., 2019). Additional resources mentioned in a review by Blomstrom et al. (2020) include psychotherapy, mindfulness, meditation, psychedelic drugs, and virtual reality. Outside of therapy, several protective factors have been identified, including increasing self-esteem (Balasubramanian et al., 2018; Greenberg et al., 1986; Greenberg et al., 1993; Greenberg et al., 1992), increasing resilience (Alagh & Chaturvedi, 2022; Azeem & Naz, 2022; Yates et al., 2015), and increasing life satisfaction (Alagh & Ghosh, 2022; Azeem & Naz, 2015; Xie & Liu, 2022). One limitation to the thanatophobia therapy research is that many studies utilize adult populations with life-threatening diseases, such as cancer (Grossman et al., 2018; Moradi et al., 2022) or AIDS/HIV (e.g., Roza et al., 2022).

While research on therapy for thanatophobia is more commonly conducted with adults, there are numerous studies regarding the use of CBT for children experiencing anxiety (Pegg et al., 2022) and grief (see Saladino et al., 2023 for a review). It is possible that a label of true thanatophobia should be reserved for adults, who understand the complexities of death. However, the anxiety and grief experienced by children could be crucial pieces in the future development of thanatophobia. Fear of death may manifest after the death of a pet, friend, or family member, if the subject of death is not properly handled by parents or caretakers at the appropriate time (Mitchell & Schulman, 1981). It is therefore logical to take steps to prevent the early onset of thanatophobia, which could begin to take root with a person's first exposure to death in childhood.

An Age Guideline on Early Thanatophobia

We suggest that treatment of thanatophobia be based primarily on age. While the following guidelines do focus on combating thanatophobia beginning in childhood, these may also be subject to the particular contexts of a person's life (as mentioned by Furer & Walker, 2008; Koocher et al., 1976; Sinoff, 2017). One such context is cognitive development. As Koocher (1973) remarks, age alone is likely not sufficient to gauge a child's ability to comprehend death, instead a measure of cognitive development is likely more fitting. Therefore, these guidelines are likely best examined as a general reference rather than an all-purpose solution for detecting and reducing early onset thanatophobia until future research suggests otherwise.

For the purposes of our general guidelines for combating thanatophobia, we have defined children as those from birth to 10 years of age based on the works of Sawyer et al. (2018) and research relating to age and death experience (e.g., Dickinson, 1992; Knight et al., 2000; Sinoff, 2017).

The basis of the following guidelines is built upon two hypotheses. First, children who have been exposed to death in a healthy manner during childhood will possess less thanatophobia later in life than children who were not. Second, adolescents and adults who suffer from thanatophobia likely suffer more from unaddressed trauma with death from childhood than adolescents and adults who do not suffer from thanatophobia. Thus, the focus of reducing thanatophobia in all age ranges is placed upon addressing the early childhood memories of death.

Guidelines for Children

It should be evident that a child's first encounter with death is the most important, as it sets the stage for all the following experiences with death. The first line of defense from thanatophobia would reside with the parents or caretakers. Four general guidelines for parents when navigating experiences with death with children are answering questions, eliminating ambiguity, acknowledging death, and uniting struggle.

Answering Questions

As previously examined, thanatophobia appears to decline with age (e.g., Koocher et al., 1976; Mohammadpour et al., 2018; Russac et al., 2007). One potential reason for this is the gaining of insight by asking questions. In general, questions are a common method to acquire knowledge about the unknown. Death is no exception. When encountering death, children are often left at the mercy of others when it comes to attaining comprehension. With some research (Dickinson, 1992) to suggest that children do process death with similar emotional reactions as adults, including disbelief, fear, and shock, children possess, at some level, an emotional understanding that the result of death is negative. Still, children often lack the logical components of why death occurs and the knowledge about death itself (Dickinson 1992; Schonfeld, 1993). While a child's first encounter with death will often occur between the ages of 7 to 9 (Dickinson, 1992; Knight et al., 2000), their understanding of death is just beginning to form during that time (Schonfeld, 1993; Sinoff, 2017). Therefore, a child's questions about death should be answered in a calm and honest manner, even if the questions seem redundant or illogical, for one should be aware that these questions are likely spawning from a lack of understanding. Children need to understand death in a healthy manner. Otherwise, it is possible for thanatophobia to begin taking its toll.

Eliminating Ambiguity

When it comes to certain topics, adults may attempt to save a child from reality by being ambiguous. For example, religion is often used as a reason to explain why someone has died (e.g., the person has "gone to Heaven"; Dickinson, 1992). Another example is that parents will state that they "will always be there for a child" (Schonfeld, 1992). While adults would understand this to be limited to the parents' lifetime, children may misconstrue the situation and misinterpret this as a form of immortality. Nonetheless, in some cases, ambiguity serves a necessary purpose to maintain innocence, such as details of how one died (e.g., murder, car accident, suicide). An excessive amount of ambiguity may eventually return as thanatophobia if the details are not eventually acknowledged in concrete terms. As mentioned by Dickinson (1992), childrens' misinterpretations of euphemisms may cause issues that are not specific to death. For example, if a deceased person is described as "sleeping" or having "gone away," sleep anxiety or vacation/trip anxiety may begin to emerge. Thus, whenever possible, parents should ensure details regarding death are as appropriate and true as possible.

Acknowledging Death

While Schonfeld (1993) argues that the discussion of death can never occur too early, it seems counterintuitive to begin prior to the age of 5, as their understanding of the dimensions of death have likely not begun. However, the subject of death should not only be mentioned when it occurs to a pet, relative, or friend. The topic may be brought up in smaller-stake cases, such as discussing reasons to not pick a flower or to not step on a bug. These steps acknowledge death in a manner that is not stressful to the child nor adult.

Uniting Struggles

When a death has occurred, it is important for a parent to try their best to process the event with children. Obviously, a time of death is an abnormal time. Grief and other negative emotions are likely present, which may lead parents to feel overwhelmed (Schonfeld, 1993). As a result of this overwhelming emotion, parents often choose to overcome the struggle of death alone by allowing relatives to take care of their children. Consequently, children's physical and emotional needs may remain unmet during this difficult time (Dickinson, 1992; Schonfeld, 1993). However, if the family is able to grieve together, not only may thanatophobia be combated, but the familial bond between parent and child may also strengthen. While death is often a time of struggle for everyone involved, it may be wise to remember the adage, "the only way out of the forest is through." Likewise with thanatophobia, the only way to prevent it is to acknowledge one of the sources of death anxiety, which is death itself.

Concluding Remarks

Everyone should seek to overcome a fear of death. However, at some level, this fear will likely be present in some form. Thus, the acceptance of death, and overcoming thanatophobia, is probably best referred to as "acquiescence"- a passive assent or an agreement without protest. Overall, thanatophobia is a multifaceted and subjective feeling. Death anxiety is handled differently across gender and culture. However, thanatophobia appears to be more consistent across age. Particularly, as a person ages, their thanatophobia tends to decrease. This potentially suggests that thanatophobia may take root in the early stages of life. Therefore, people should seek to combat thanatophobia during childhood. However, it is a heavy responsibility for parents to ensure a child's first encounter with death is as positive as it can be, for parents are often one of the only lines of defense a child possesses in the turbulent time that death creates. While several therapies have been tested in regard to lowering thanatophobia in adults, there are limited empirical studies examining thanatophobia in children. This paper therefore proposes guidelines based on the theories discussed to potentially help reduce any early onset thanatophobia. Questions should be answered truthfully with an avoidance of ambiguous answers. Moreover, death should be acknowledged as a function of life. It is not inherently good nor evil, but it is also not something one should fear. And, during an encounter with death, it is critically important to fight through it together as a family. Death is an abnormal time, but the turmoil of death may be a better and healthier experience for all when it is processed together. It is these early points in time that likely sets the stage for overcoming or succumbing to thanatophobia later in life. Therefore, thanatophobia must be stopped early before it has the potential to take root. And, hopefully, it then becomes possible for an individual to reach acquiescence and avoid the onset of thanatophobia.

Statements and Declarations - The author reports no conflict of interest. Ethical approval was obtained by the respective institutional review board prior to data collection.

Funding - This work was not funded by any agency or entity.

References

- Alagh, T., & Chaturvedi, R. (2022). Death anxiety and resilience in elderly. *NeuroQuantology*, 20(17), 7-11.
- Alagh, T., & Ghosh, S. (2022). Impact of fear of personal death on life satisfaction among old age people. *Journal of Positive School Psychology*, 401-408.
- Azeem, F., & Naz, M. A. (2015). Resilience, death anxiety, and depression among institutionalized and noninstitutionalized elderly. *Pakistan Journal of Psychological Research*, 30(1), 111-130.
- Balasubramanian, C., Subramanian, M., Balasubramanian, S., Agrawal, A., Raveendran, S., & Kaliaperumal, C. (2018). "Thanatophobia": Physician's perspective of dealing with patients with fear of death. *Journal of Natural Science, Biology, and Medicine*, 9(1), 103-104.
- Blomstrom, M., Burns, A., Larriviere, D., & Penberthy, J. K. (2022). Addressing fear of death and dying: traditional and innovative interventions. *Mortality*, 27(1), 18-37. <https://doi.org/10.1080/13576275.2020.1810649>
- Çakar, F. S. (2020). The levels predicting the death anxiety of loneliness and meaning in life in youth. *European Journal of Education Studies*, 6(11). <http://dx.doi.org/10.5281/zenodo.3626703>

- Chen, C. Y. C., & Panebianco, A. (2018). Interventions for young bereaved children: A systematic review and implications for school mental health providers. In *Child & Youth Care Forum* (Vol. 47, pp. 151-171). Springer US. <https://doi.org/10.1007/s10566-017-9426-x>
- Chopik, W. J. (2017). Death across the lifespan: Age differences in death-related thoughts and anxiety. *Death Studies*, 41(2), 69-77. <https://doi.org/10.1080/07481187.2016.1206997>
- Dickinson, G. E. (1992). First childhood death experiences. *Omega-Journal of Death and Dying*, 25(3), 169-182. <https://doi.org/10.2190/M8F0-TN3F-EFCB-H8N3>
- Furer, P., & Walker, J. R. (2008). Death anxiety: A cognitive-behavioral approach. *Journal of Cognitive Psychotherapy*, 22(2), 167-182. <https://doi.org/10.1891/0889-8391.22.2.167>
- Greenberg, J., Pyszczynski, T., & Solomon, S. (1986). The causes and consequences of a need for self-esteem: A terror management theory. *Public Self and Private Self*, 189-212. Springer Series in Social Psychology. Springer, New York, NY. https://doi.org/10.1007/978-1-4613-9564-5_10
- Greenberg, J., Pyszczynski, T., Solomon, S., Pinel, E., Simon, L., & Jordan, K. (1993). Effects of self-esteem on vulnerability-denying defensive distortions: Further evidence of an anxiety-buffering function of self-esteem. *Journal of Experimental Social Psychology*, 29(3), 229-251. <https://doi.org/10.1006/jesp.1993.1010>
- Greenberg, J., Solomon, S., Pyszczynski, T., Rosenblatt, A., Burling, J., Lyon, D., ... & Pinel, E. (1992). Why do people need self-esteem? Converging evidence that self-esteem serves an anxiety-buffering function. *Journal of Personality and Social Psychology*, 63(6), 913-922. <https://doi.org/10.1037/0022-3514.63.6.913>
- Grossman, C. H., Brooker, J., Michael, N., & Kissane, D. (2018). Death anxiety interventions in patients with advanced cancer: A systematic review. *Palliative Medicine*, 32(1), 172-184. <https://doi.org/10.1177/026921631772212>
- Iverach, L., Menzies, R. G., & Menzies, R. E. (2014). Death anxiety and its role in psychopathology: Reviewing the status of a transdiagnostic construct. *Clinical Psychology Review*, 34(7), 580-593. <https://doi.org/10.1016/j.cpr.2014.09.002>
- Kesebir, P. (2014). A quiet ego quiets death anxiety: Humility as an existential anxiety buffer. *Journal of Personality and Social Psychology*, 106(4), 610-623. <https://doi.org/10.1037/a0035814>
- Knight, K. H., Elfenbein, M. H., & Capozzi, L. (2000). Relationship of recollections of first death experience to current death attitudes. *Death Studies*, 24(3), 201-221. <https://doi.org/10.1080/074811800200540>
- Koocher, G. P. (1973). Childhood, death, and cognitive development. *Developmental psychology*, 9(3), 369-375. <https://doi.org/10.1037/h0034917>
- Koocher, G. P., O'Malley, J. E., Foster, D., & Gogan, J. L. (1976). Death anxiety in normal children and adolescents. *Psychopathology*, 9(3-4), 220-229. <https://doi.org/10.1159/000283682>
- Krause, N., & Bastida, E. (2012). Contact with the dead, religion, and death anxiety among older Mexican Americans. *Death Studies*, 36(10), 932-948. <https://doi.org/10.1080/07481187.2011.604468>
- Lehto, R. H., & Stein, K. F. (2009). Death anxiety: an analysis of an evolving concept. *Research and theory for nursing practice*, 23(1), 23-41. <https://doi.org/10.1891/1541-6577.23.1.23>
- Menzies, R. E., Sharpe, L., Helgadóttir, F. D., & Dar-Nimrod, I. (2021). Overcome Death Anxiety: The Development of an Online Cognitive Behaviour Therapy Programme for Fears of Death. *Behaviour Change*, 38(4), 235-249. <https://doi.org/10.1017/bec.2021.14>
- Mitchell, N. L., & Schulman, K. R. (1981). The child and the fear of death. *Journal of the National Medical Association*, 73(10), 963.
- Mohammadpour, A., Sadeghmoghadam, L., Shareinia, H., Jahani, S., & Amiri, F. (2018). Investigating the role of perception of aging and associated factors in death anxiety among the elderly. *Clinical Interventions in Aging*, 405-410. <https://doi.org/10.2147/CIA.S150697>
- Moradi, M., Akbari, M., & Alavi, M. (2022). The effect of cognitive-behavioral therapy on death anxiety and depression in patients with heart failure: A quasi-experimental study. *Perspectives in Psychiatric Care*, 58(4), 2791-2799. <https://doi.org/10.1111/ppc.13125>
- Nozari, Z., Mo'tamedi, A., Eskandari, H., & Ahmadiwand, Z. (2019). Narrative group therapy to improve aging perceptions and reduce thanatophobia (death anxiety) in older adults. *Elderly Health Journal*. <https://doi.org/10.18502/ehj.v5i2.2158>
- Pegg, S., Hill, K., Argiros, A., Olatunji, B. O., & Kujawa, A. (2022). Cognitive Behavioral Therapy for Anxiety Disorders in Youth: Efficacy, Moderators, and New Advances in Predicting Outcomes. *Current Psychiatry Reports*, 24(12), 853-859. <https://doi.org/10.1007/s11920-022-01384-7>

- Roza, D., Yanti, N., Suryarini, Y., Alfitri, A., & Sasmita, H. (2022). Cognitive Behavior Therapy (CBT) to Reduce Death Anxiety (Thanatophobia) in HIV/AIDS Patients. *Jurnal Aisyah: Jurnal Ilmu Kesehatan*, 7(4), 1131-1138. <https://doi.org/10.30604/jika.v7i4.1320>
- Russac, R. J., Gatliff, C., Reece, M., & Spottswood, D. (2007). Death anxiety across the adult years: An examination of age and gender effects. *Death Studies*, 31(6), 549–561. <https://doi.org/10.1080/07481180701356936>
- Saladino, V., Verrastro, V., Calaresi, D., & Barberis, N. (2023). The effectiveness of cognitive behavioral therapy for prolonged grief symptoms in children and adolescents: A systematic review. *International Journal of Stress Management*. <https://doi.org/10.1037/str0000301>
- Sawyer, S. M., Azzopardi, P. S., Wickremarathne, D., & Patton, G. C. (2018). The age of adolescence. *The Lancet Child & Adolescent Health*, 2(3), 223-228. [https://doi.org/10.1016/S2352-4642\(18\)30022-1](https://doi.org/10.1016/S2352-4642(18)30022-1)
- Schonfeld, D. J. (1993). Talking with children about death. *Journal of Pediatric Health Care*, 7(6), 269-274. [https://doi.org/10.1016/s0891-5245\(06\)80008-8](https://doi.org/10.1016/s0891-5245(06)80008-8)
- Schumaker, J. F., Warren, W. G., & Groth-Marnat, G. (1991). Death anxiety in Japan and Australia. *The Journal of Social Psychology*, 131(4), 511-518. <https://doi.org/10.1080/00224545.1991.9713881>
- Seidi, P. A., & Jaff, D. (2023). Cognitive behavioral therapy and death anxiety: A case of children in conflict settings during the COVID-19 pandemic. In *Handbook of Lifespan Cognitive Behavioral Therapy* (pp. 459-464). Academic Press. <https://doi.org/10.1016/B978-0-323-85757-4.00010-9>
- Sinoff, G. (2017). Thanatophobia (death anxiety) in the elderly: The problem of the child's inability to assess their own parent's death anxiety state. *Frontiers in Medicine*, 11. <https://doi.org/10.3389/fmed.2017.00011>
- Suhail, K., & Akram, S. (2002). Correlates of death anxiety in Pakistan. *Death Studies*, 26(1), 39-50. <https://doi.org/10.1080/07481180210146>
- World Health Organization. (1965). *Health problems of adolescence: report of a WHO Expert Committee [meeting held in Geneva from 3 to 9 November 1964]*. World Health Organization.
- World Health Organization. (2018). *The Global Strategy for Women's, Children's and Adolescents' Health (2016-2030)*. [https://www.who.int/publications/i/item/the-global-strategy-for-women-s-children-s-and-adolescents-health-\(2016-2030\)-early-childhood-development-report-by-the-director-general](https://www.who.int/publications/i/item/the-global-strategy-for-women-s-children-s-and-adolescents-health-(2016-2030)-early-childhood-development-report-by-the-director-general).
- Yates, T. M., Tyrell, F. A., & Masten, A. S. (2015). Resilience theory and the practice of positive psychology from individuals to societies. *Positive Psychology in Practice: Promoting Human Flourishing in Work, Health, Education, and Everyday Life*, 773-788. <https://doi.org/10.1002/9781118996874.ch44>
- Xie, Y., & Liu, B. (2022). Life satisfaction and death anxiety among Chinese rural elderly: Moderating effects of gender, age and spouse.
- Zuccala, M., & Abbott, M. J. (2021). Social anxiety disorder and the fear of death: An empirical investigation of the terror management approach towards understanding clinical anxiety. *Cognitive Therapy and Research*, 45(4), 628–641. <https://doi.org/10.1007/s10608-020-10187-0>